

IN THE HOUSE OF REPRESENTATIVES

HOUSE BILL NO. 806

BY HEALTH AND WELFARE COMMITTEE

AN ACT

RELATING TO PHYSICIANS AND PHYSICIAN ASSISTANTS; AMENDING SECTION 54-1807A, IDAHO CODE, TO REVISE PROVISIONS REGARDING THE PRACTICE OF PHYSICIAN ASSISTANTS; AMENDING SECTION 54-1814, IDAHO CODE, TO PROVIDE FOR HEALTH PROFESSIONALS WHO REQUIRE SUPERVISION; AMENDING SECTION 54-1867, IDAHO CODE, TO REMOVE A LIMITATION; AND DECLARING AN EMERGENCY AND PROVIDING AN EFFECTIVE DATE.

Be It Enacted by the Legislature of the State of Idaho:

SECTION 1. That Section 54-1807A, Idaho Code, be, and the same is hereby amended to read as follows:

54-1807A. PHYSICIAN ASSISTANTS -- PHYSICIAN ASSISTANT ADVISORY COMMITTEE. (1) Physician assistants must be licensed by the board prior to the commencement of activities that may involve the practice of medicine in this state. The licensure requirements for physician assistants shall include passage of an examination acceptable to the board and submission of a completed application to the board on forms furnished by the board. All applicants for original licensure as a physician assistant shall submit to a fingerprint-based criminal history check in accordance with section 67-9411A, Idaho Code. All physician assistants shall renew their licenses biennially.

(2) A physician assistant may practice at a licensed health care facility, a facility with a credentialing and privileging system, a physician-owned facility or practice, or another facility or practice allowed by this chapter. A physician assistant shall collaborate with, consult with, or refer to the appropriate member of the facility health care team as indicated by: ~~the condition of the patient; the education, experience, and competence of the physician assistant; and the community standard of care. The degree and nature of collaboration shall be determined by the facility or practice in which the physician assistant works and shall be set forth in facility bylaws or procedures for facilities that have credentialing and privileging systems or in a written collaborative practice agreement for all other facilities and practices. Such provisions may provide for collaborative oversight to be provided by the employer, group, hospital service, or the credentialing and privileging systems of a licensed facility, but at a minimum shall require a physician assistant to collaborate with one (1) or more physicians licensed pursuant to this chapter. Such physicians need not be identified individually in the facility bylaws or procedures or collaborative practice agreement if more than one (1) physician works in the facility or practice. A physician assistant is responsible for the care provided by the physician assistant and is responsible for obtaining professional liability insurance if the physician assistant is not covered by the facility or practice in which the physician assistant works. A physician assistant may be employed by nonphysician health care providers if the~~

1 physician assistant has a written collaborative practice agreement with one
 2 (1) or more physicians licensed pursuant to this chapter. Both the physi-
 3 cian assistant and the physician who are parties to a collaborative practice
 4 agreement must comply with all requirements of this chapter and board rules.
 5 The collaborative practice agreement shall be provided to the board upon re-
 6 quest. necessary to appropriately assess and treat the condition of the pa-
 7 tient within the scope of the physician assistant's education, training, and
 8 experience and within the community standard of care. The degree and nature
 9 of collaboration shall be determined by the facility or practice in which
 10 the physician assistant works and shall be set forth in facility bylaws or
 11 procedures for facilities that have credentialing and privileging systems
 12 or in a written collaborative practice agreement for all other facilities
 13 and practices. Such provisions may provide for collaborative oversight to
 14 be provided by the employer, group, hospital service, or the credentialing
 15 and privileging systems of a licensed facility. A physician assistant is
 16 responsible for the care provided and is responsible for obtaining profes-
 17 sional liability insurance if the physician assistant is not covered by the
 18 facility or practice in which the physician assistant works. A physician
 19 assistant may be employed by nonphysician health care providers. Any col-
 20 laborative practice agreement shall be provided to the board upon request.

21 (3) A physician assistant or a group of physician assistants may inde-
 22 pendently own a medical practice in this state provided that each physician
 23 assistant has a collaborative practice agreement in place with a physician
 24 licensed under this chapter. The collaborative practice agreement shall
 25 specify that the physician assistant must collaborate with, consult with,
 26 or refer to the collaborating physician or another appropriate physician
 27 as indicated by: the condition of the patient; the education, experience,
 28 and competence of the physician assistant; and the community standard of
 29 care. Both the physician assistant and the physician who are parties to
 30 the collaborative practice agreement must comply with all requirements of
 31 this chapter and board rules. The collaborative practice agreement shall
 32 be provided to the board upon request. A physician assistant or group of
 33 physician assistants may independently own a medical practice in this state.
 34 Any collaborative practice agreement shall be provided to the board upon
 35 request. Each physician assistant must be licensed, registered or certi-
 36 fied as a physician assistant in any state, territory or jurisdiction of the
 37 United States for at least two (2) years before the physician assistant may
 38 independently own a practice in this state.

39 (4) The facility or practice and each collaborating physician are re-
 40 sponsible for ensuring that the medical services performed by the physician
 41 assistant are within the physician assistant's scope of education, experi-
 42 ence, and competence. Each collaborating physician shall collaborate with
 43 the physician assistant on the performance of only those medical services
 44 for which the collaborating physician has training and experience.

45 (5) (4) A physician assistant advisory committee is hereby established
 46 as follows:

- 47 (a) The physician assistant advisory committee shall consist of five
- 48 (5) members appointed by the board. Four (4) members shall be physician
- 49 assistants who are residents in this state and engaged in the active
- 50 practice of medicine in this state, and one (1) member shall be a public

1 member. Whenever a term of a member of the advisory committee expires or
 2 becomes vacant, the board shall give consideration to recommendations
 3 made by professional organizations of physician assistants and physi-
 4 cians or by any individual residing in the state. The board may remove
 5 any committee member for misconduct, incompetency or neglect of duty
 6 after giving the member a written statement of the charges and an oppor-
 7 tunity to be heard thereon. The executive director of the Idaho state
 8 board of medicine shall serve as the executive director to the physician
 9 assistant advisory committee.

10 (b) Members will serve a term of three (3) years and terms will be stag-
 11 gered. Members may serve two (2) successive terms. The committee shall
 12 elect a chairman from its membership. The committee shall meet as often
 13 as necessary to fulfill its responsibilities. Members will be compen-
 14 sated according to section 59-509(p), Idaho Code.

15 (c) The physician assistant advisory committee shall not have author-
 16 ity to revoke licenses or impose limitations or conditions on licenses
 17 issued pursuant to this chapter. The committee has authority to make
 18 recommendations to the board. The board shall make all final decisions
 19 with respect thereto.

20 (d) The physician assistant advisory committee shall work in the fol-
 21 lowing areas in conjunction with and make recommendations to the board
 22 and shall perform other duties and functions assigned to it by the
 23 board, including:

- 24 (i) Evaluating the qualifications of applicants for licensure
- 25 and registration;
- 26 (ii) Performing investigations of misconduct and making recom-
- 27 mendations regarding discipline;
- 28 (iii) Maintaining a list of currently licensed physician assis-
- 29 tants and graduate physician assistants in this state; and
- 30 (iv) Advising the board on rule changes necessary to license and
- 31 regulate physician assistants and graduate physician assistants
- 32 in this state.

33 SECTION 2. That Section 54-1814, Idaho Code, be, and the same is hereby
 34 amended to read as follows:

35 54-1814. GROUNDS FOR MEDICAL DISCIPLINE. Every person licensed to
 36 practice medicine, or registered as an intern or resident in this state is
 37 subject to discipline by the board pursuant to the procedures set forth in
 38 this chapter and rules promulgated pursuant thereto upon any of the follow-
 39 ing grounds:

- 40 (1) Being convicted of a felony, pleading guilty to a felony, or the
- 41 finding of guilt by a jury or court of commission of a felony.
- 42 (2) Using false, fraudulent or forged statements or documents, diplo-
- 43 mas or credentials in connection with any licensing or other requirements of
- 44 this act.
- 45 (3) Practicing medicine under a false or assumed name in this or any
- 46 other state.
- 47 (4) Advertising the practice of medicine in any unethical or unprofes-
- 48 sional manner.

1 (5) Knowingly aiding or abetting any person to practice medicine who is
2 not authorized to practice medicine as provided in this chapter.

3 (6) Performing or procuring an unlawful abortion or aiding or abetting
4 the performing or procuring of an unlawful abortion.

5 (7) Providing health care which fails to meet the standard of health
6 care provided by other qualified physicians or physician assistants in the
7 same community or similar communities, taking into account his training, ex-
8 perience and the degree of expertise to which he holds himself out to the pub-
9 lic.

10 (8) Dividing fees or gifts or agreeing to split or divide fees or gifts
11 received for professional services with any person, institution or corpora-
12 tion in exchange for referral.

13 (9) Giving or receiving or aiding or abetting the giving or receiving of
14 rebates, either directly or indirectly.

15 (10) Inability to obtain or renew a license to practice medicine, or re-
16 vocation, suspension, or other discipline of a license to practice medicine
17 by any other state, territory, district of the United States or Canada, un-
18 less it can be shown that such action was not related to the competence of the
19 person to practice medicine or to any conduct designated herein.

20 (11) Prescribing or furnishing narcotic or hallucinogenic drugs to ad-
21 dicted persons to maintain their addictions and level of usage without at-
22 tempting to treat the primary condition requiring the use of narcotics.

23 (12) Prescribing or furnishing narcotic, hypnotic, hallucinogenic,
24 stimulating or dangerous drugs for other than treatment of any disease, in-
25 jury or medical condition.

26 (13) Failing to safeguard the confidentiality of medical records or
27 other medical information pertaining to identifiable patients, except as
28 required or authorized by law.

29 (14) Directly promoting the sale of drugs, devices, appliances or goods
30 to a patient that are unnecessary and not medically indicated.

31 (15) Abandoning a patient.

32 (16) Willfully and intentionally representing that a manifestly incur-
33 able disease or injury or other manifestly incurable condition can be perma-
34 nently cured.

35 (17) Failing to supervise the activities of ~~interns, residents, nurse~~
36 ~~practitioners, certified nurse-midwives, clinical nurse specialists, or~~
37 ~~physician assistants~~ health professionals who require supervision.

38 (18) Practicing medicine when a license pursuant to this chapter is sus-
39 pended, revoked or inactive.

40 (19) Practicing medicine in violation of a voluntary restriction or
41 terms of probation pursuant to this chapter.

42 (20) Refusing to divulge to the board upon demand the means, method, de-
43 vice or instrumentality used in the treatment of a disease, injury, ailment,
44 or infirmity.

45 (21) Committing any act constituting a felony.

46 (22) Engaging in any conduct which constitutes an abuse or exploitation
47 of a patient arising out of the trust and confidence placed in the physician
48 by the patient.

1 (23) Being convicted of or pleading guilty to driving under the influ-
 2 ence of alcohol, drugs or other intoxicating substances or being convicted
 3 of or pleading guilty to other drug or alcohol related criminal charges.

4 (24) Failing to comply with a board order entered by the board.

5 (25) Failing to comply with the requirements of the abortion complica-
 6 tions reporting act, chapter 95, title 39, Idaho Code.

7 (26) Engaging in a pattern of unprofessional or disruptive behavior or
 8 interaction in a health care setting that interferes with patient care or
 9 could reasonably be expected to adversely impact the quality of care ren-
 10 dered to a patient. Such behavior does not have to have caused actual patient
 11 harm to be considered unprofessional or disruptive.

12 (27) Interfering with an investigation or disciplinary proceeding
 13 by willful misrepresentation of facts or by use of threats of harassment
 14 against any patient, member of a board or committee on professional disci-
 15 pline, board staff, hearing officer, or witness in an attempt to influence
 16 the outcome of a disciplinary proceeding, investigation, or other legal
 17 action.

18 (28) Delegating professional responsibilities to:

19 (a) An unlicensed person when the licensee knows or has reason to know
 20 that such person is not qualified by training, experience, or license to
 21 carry them out; or

22 (b) A person licensed by this state to engage in activities which may
 23 involve the practice of medicine when the delegating licensee knows or
 24 has reason to know that the delegated activities are outside the li-
 25 censed person's scope of practice.

26 (29) Failure to report the charge or conviction of a felony to the board
 27 within thirty (30) days of the charge.

28 SECTION 3. That Section 54-1867, Idaho Code, be, and the same is hereby
 29 amended to read as follows:

30 54-1867. LIMITED LICENSE FOR BRIDGE YEAR PHYSICIANS. (1) As used in
 31 this section:

32 (a) "Board" means the state board of medicine.

33 (b) "Bridge year physician" means a person who:

34 (i) Is within the first year of graduation from a medical school
 35 accredited or provisionally accredited by an entity recognized by
 36 the board;

37 (ii) Is a United States citizen or attended medical school in the
 38 United States; and

39 (iii) Applied to, but was not accepted into, an accredited medical
 40 residency training program.

41 (2) The board shall establish a one (1) year, nonrenewable limited li-
 42 cense under which bridge year physicians may practice medicine under terms,
 43 conditions, and a scope of practice determined by the board. If there is a
 44 limit to the number of limited licenses that may be issued, priority shall be
 45 granted to bridge year physicians who live in Idaho or who have longstanding
 46 ties to the state of Idaho, as determined by the board.

47 (3) Persons practicing under a limited license established pursuant to
 48 this section shall:

49 (a) Practice only within the scope of practice determined by the board;

(b) Practice under the supervision of a licensed physician or pursuant to a collaborative practice agreement. The person practicing under a limited license shall qualify as one (1) of a supervising physician's permitted advanced practice professionals. The board shall prescribe supervision requirements for limited licensees, ~~provided that such requirements shall be no less stringent than supervision requirements for physician assistants;~~

(c) Have prescriptive authority as determined by the board; and

(d) Be subject to the same professional discipline, civil liability, and criminal liability as a fully licensed physician.

(4) The services provided by a person practicing under a limited license shall be compensable in accordance with customary medical billing practices.

(5) The board is authorized to:

(a) Take such actions as are necessary to implement the provisions of this section, including the promulgation of any necessary rules;

(b) Charge a fee of up to three hundred dollars (\$300) for a limited license; and

(c) Cooperate with the department of health and welfare and other relevant entities, including hospitals and health care clinics, whether public or private, in establishing a limited license.

(6) No later than January 31, 2033, the board shall provide a report to the senate and house of representatives health and welfare committees on:

(a) Requirements for a limited license;

(b) The number of limited licenses issued and the number of limited license holders who were later accepted into a residency program; and

(c) Whether and how limited licenses have increased the supply of health care providers in health professional shortage areas.

SECTION 4. An emergency existing therefor, which emergency is hereby declared to exist, this act shall be in full force and effect on and after July 1, 2026.